

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-32A25C6570

Country: Burkina Faso

Submission Date: 2025-04-26

Product: Atenra (atenolol) | ATC: C07AB03 | Strength: 100 mg

Applicant: Prime Therapeutics | Manufacturer: Mwanza Pharma Industries

Policy Label: fast_track | Risk Score: 0.28

AMR Stewardship: AWaRe=not_applicable | Unmet Need=not_applicable | GLASS Trend=not_applicable
| Watch Similarity=not_applicable

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

This dossier constitutes a formal application for Marketing Authorization of Atenra (atenolol) submitted on 2025-04-26 to the national regulatory authority of Burkina Faso. The applicant, Prime Therapeutics, formally requests approval for the commercial distribution of this medicinal product. The proposed pharmaceutical form is tablet with a strength of 100 mg, classified under ATC code C07AB03. All required administrative declarations, legal attestations, letters of authorization, and regional application forms have been completed, signed by the responsible Qualified Person, and appended to Module 1. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The proposed prescribing information and patient information leaflet have been translated into local languages. Method validation records and quality controls were reviewed for internal consistency. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. A comprehensive table of contents and glossary of abbreviations are provided to facilitate regulatory review. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Regional administrative forms were checked for signature completeness and dossier version alignment. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Administrative cover letters, declarations, and fee records were cross-checked against the submission index. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The applicant confirms alignment with applicable technical guidance and regional submission standards. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professi

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=correct

The primary commercial manufacturing site for the finished pharmaceutical product is Mwanza Pharma Industries, located in Botswana. The facility was subject to a comprehensive GMP inspection by a recognized stringent regulatory authority. The latest inspection concluded on 2024-10-13, resulting in a formal compliance status of 'compliant'. The active GMP certificate number is GMP-389930, which remains valid until 2027-06-26. Site master files, a summary of recent inspection observations, and evidence of completed Corrective and Preventive Actions (CAPA) are included in the annex. The GMP evidence package includes certificate status, inspection scope, and manufacturing-site accountability records. Statistical significance was set at an alpha level of 0.05, and all tests were two-side

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=correct

The proposed proprietary name for this medicinal product is Atenra, containing the active pharmaceutical ingredient atenolol. The draft Summary of Product Characteristics (SmPC), Patient Information Leaflet (PIL), and primary/secondary packaging labels are provided for review. These documents detail the approved therapeutic indications, posology, contraindications, special warnings, and precautions for use. A comprehensive risk management plan to minimize medication errors is also submitted. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Method validation records and quality controls were reviewed for internal consistency. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for atenolol. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. All referenced documents are available in the dossier annex and traceable by section identifier. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Adverse events were coded using the Medical Di

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=correct

The Clinical Overview synthesizes the efficacy and safety data supporting the use of Atenra for the treatment of bacterial skin infection. The pivotal clinical development program comprised 4 well-controlled studies. Based on the integrated analysis, the reported clinical outcome category is 'endpoint_met'. The overall benefit-risk profile is considered highly favorable for the target patient population. Safety findings were consistent with the known pharmacological class effects, and appropriate risk minimization measures have been integrated into the proposed prescribing information. The clinical overview links the therapeutic rationale to the pivotal evidence package and the proposed indication. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. No new safety signals were identified during the extended open-label follow-up period. The target population demographics are broadly representative of the intended commercial patient cohort. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Adverse events were coded using the Medical Dictionary for Regulator

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Critical material attributes and control strategy elements were reviewed for consistency across the quality dossier. All referenced documents are available in the dossier annex and traceable by section identifier. The applicant confirms alignment with applicable technical guidance and regional submission standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Batch analysis, validation, and impurity management records support the proposed API quality framework. Method validation records and quality controls were reviewed for internal consistency. Residual solvents and elemental impurities are controlled well below ICH Q3C and Q3D permissib

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-time studies, and packaging validation data are presented to substantiate the robustness and reproducibility of the commercial manufacturing operations. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Method validation records and quality controls were reviewed for internal consistency. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The applicant confirms alignment with applicable technical guidance and regional submission standards. All referenced documents are available in the dossier annex and traceable by section identifier. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Risk mitigation acti

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage

conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The applicant confirms alignment with applicable technical guidance and regional submission standards. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Method validation records and quality controls were reviewed for internal consistency. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. All referenced documents are available in the dossier annex and traceable by section identifier. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The risk management plan includes routine pharmacovigilance and proposed educational m

[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Data were analyzed using a mixed-effects model for repeated measures, adj

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

The tabular listing of clinical studies provides a comprehensive inventory of all trials conducted in support of the proposed indication (bacterial skin infection). This includes Phase I pharmacokinetic/pharmacodynamic studies, Phase II dose-finding studies, and the pivotal Phase III efficacy and safety trials. For each study, the table details the protocol identifier, study design, randomization ratio, number of enrolled subjects, study duration, and current completion status. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The applicant confirms alignment with applicable technical guidance and regional submission standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating

acceptable accuracy, precision, and specificity. Data were analyzed using a mixed-effects

[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=present; length=length_ok; correctness=correct

Detailed clinical study reports (CSRs) for the pivotal efficacy trials are provided. These randomized, double-blind, actively-controlled trials evaluated the primary and secondary endpoints in accordance with the pre-specified statistical analysis plan (SAP). The trials resulted in a formal outcome category of 'endpoint_met', demonstrating robust treatment effects in the intent-to-treat (ITT) and per-protocol populations. Subgroup analyses across age, gender, and baseline disease severity strata were consistent with the primary findings. Serious adverse events (SAEs) and discontinuations were systematically analyzed and adjudicated. The primary efficacy endpoint was met, demonstrating a statistically significant improvement over placebo ($p < 0.001$). Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. All referenced documents are available in the dossier annex and traceable by section identifier. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The trial reports connect efficacy conclusions to the prespecified analysis framework and observed safety findings. Clinical study reports document protocol adherence, endpoint handling, and safety interpretation in the target population. The pivotal study narratives align endpoint definitions, analysis populations, and outcome interpretation. Method validation records and quality controls were reviewed for internal consistency. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Discontinuations due to treatment-emergent adverse events (TEAEs) were low and balanced across all study arms. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The applicant confirms alignment with applicable technical guidance and regional submission standards. The trial reports connect efficacy conclusions to the prespecified analysis framework and observed safety findings. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Clinical study reports document protocol adherence, endpoint handling, and safety interpretation in the target population. Pharmacokinetic parameters were derived using non-compartmental analysis.

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=length_ok; correctness=correct

Biopharmaceutics data substantiate the formulation development bridging between clinical trial materials and the proposed commercial product. In vivo bioequivalence studies demonstrated that the 90% confidence intervals for the geometric mean ratios of C_{max} and AUC parameters fell entirely within the standard acceptance criteria of 80.00% to 125.00%. Additionally, multi-point comparative dissolution profiles across physiological pH ranges (pH 1.2, 4.5, and 6.8) confirmed similarity ($f_2 > 50$). The bioanalytical methods used for plasma quantification were fully validated. The risk management plan includes routine pharmacovigilance and proposed

educational materials for healthcare professionals. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. All referenced documents are available in the dossier annex and traceable by section identifier. Method validation records and quality controls were reviewed for internal consistency. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The applicant confir